

Private Practice Psychiatry, Weekly Digest

Week of August 1 to August 7, 2026 | 66 posts reviewed | Generated August 8, 2026

Source: Private Practice Psychiatry Facebook group, read-only. Themes and paraphrase only. No author names, no patient-identifying detail. Roughly 25 of this week's 66 posts were "looking for a psychiatrist in X" referral requests and are excluded as noise.

The story of the week: insurers are downcoding 99214s, and doctors are winning them back

This was the loudest thread in the group by a wide margin, and it is the one item here with a directly usable playbook.

One Illinois psychiatrist reported submitting notes on **every** downcoded 99214 and getting a **100% reversal rate**, with the claims returned and paid at the correct level. The more interesting part: **one week after starting to appeal, all new 99214s began paying on first pass**. Their read was that appealing changes payer behavior, not just the individual claim. They described it as annoying but not painful.

A second member asked what language works, and reported already using explicit MDM-versus-time framing plus level-of-complexity language for each claim.

Sentiment has escalated past appeals. One member asked whether the group should consider a **class action against BCBS**, framing the downcoding as breach of contract. Another, working in an insurance-based practice, asked whether a **monthly administrative fee** for unbillable work (parent calls, therapist coordination, forms, between-visit email) could be structured legally.

What this means for JP: if any payer has been quietly downcoding, the reported pattern is that a consistent appeal habit pays for itself twice, once on the recovered claims and again on future first-pass behavior. Worth a look at whether JP's 99214s are being paid at the submitted level, and whether anyone is checking. This is the highest-value item in the digest.

Insurance and billing

- **PHQ-2 is separately billable.** A member discovered it codes separately at roughly a \$10 blended rate and said they will now administer it universally. Low effort, recurring, per visit.
- **Aetna telemedicine coding is unclear to practitioners.** An open question on which POS and modifier Aetna expects for E/M telemedicine, or whether they force the newer telemedicine codes. No resolution in the post.

- **Medicare participating versus non-participating** enrollment came up again, pros and cons requested, unresolved.
- **Tricare and cash practices:** whether fully cash, non-credentialed practices need a Tricare waiver the way opted-out Medicare requires. Unanswered, and worth knowing definitively rather than guessing.
- **A compliance problem worth noting as a cautionary tale:** a member described a colleague who collected cash from patients who may have had insurance, while credentialed with inpatient units, some years back. The group treated this as a serious exposure. The lesson others drew was that the rule is not widely known.
- **Coding edge case:** how to code a lengthy intake for a nonverbal patient with complex medical history, an hour of collateral, and heavy care coordination. The question was whether 99205 alone is right.
- **Optum pharmacy friction:** repeated "you need to authorize the prescription" responses on already-transmitted prescriptions, with the payer denying any problem when called. Described as new after years without issue.

EHR and platform

- **Charm is failing loudly.** One member detailed giving up on PDMP integration entirely, then signing up for e-claims and having nothing work: **over 40 claims stuck unprocessed**, daily calls for a week, a strongly worded email, perpetual "escalating the issue," and zero communication on timeline. Their biller has experience with the platform, so this does not read as user error. **If JP ever evaluates Charm, this is a live data point.**
- **Phone line necessity questioned.** A member starting a cash practice asked whether a dedicated HIPAA-compliant phone line is strictly necessary given SimplePractice messaging for established patients plus Doximity for return calls. The underlying question is what patients expect versus what compliance requires.
- **Agentic AI for front office.** A member asked whether anyone has explored **Claude Cowork or similar agentic AI** for practice administration: phone calls, scheduling, referrals. Their framing was not replacing staff but reducing the training and retraining burden and the variability in what staff deliver. Notable both as a market signal and because it is the problem JP already solves differently.

Business operations

- **Malpractice coverage has two open questions this week.** First, whether **\$1M per occurrence / \$3M aggregate** is standard or low for psychiatry, asked after a healthcare lawyer raised it. Second, and more practically, **whether malpractice covers you when providing vacation coverage for another practitioner.** Informal cross-coverage between community psychiatrists appears to be common and the insurance treatment of it is unclear to the people doing it.

- **DEA registration will not accept a virtual mailbox.** A Texas member is seeking a colleague willing to let them use a physical office address for DEA registration. A real structural constraint for virtual-first practices.
- **Health insurance for the self-employed** resurfaced, with marketplace premiums called exorbitant, especially with dependents and especially for plans with decent coverage. Asked once before with limited response.
- **Billers are a pain point.** One member is struggling with theirs and asked for referrals, specifying private insurance plus Medi-Cal and Medi-Cal partnership in California.
- **Self-audits.** A member asked whether anyone runs regular practice self-audits and would share a checklist or approach. No responses captured. This is the kind of thing worth having before someone else audits you.
- **Marketing beyond word of mouth,** asked by someone leaving a 20-year hospital career for private practice.
- **Documentation templates,** from a child and adolescent psychiatrist starting out: what daily templates for intakes and follow-ups look like, and how rigidly people actually stick to them.
- **Adjacent income streams** appeared three times: medical review officer work for drug screens, peer-to-peer review work against insurers for hospitalized patients, and virtual IOP/PHP roles.
- **Two softer ops threads:** navigating the discomfort of declining a patient who is not a good fit, and a request for mentorship from an early-career psychiatrist opening in Oakland. Also one member's one-year solo practice anniversary, which drew warm response.

Clinical intelligence

- **CVS and stimulant refills, the most actionable clinical item.** A member reported CVS refusing to fill a stimulant prescription for a long-term, stable patient, and being told that **company policy shifted in October 2025 to require a urine drug screen for every patient at every CVS nationwide before a stimulant refill.** When they asked where the policy is published and why they were hearing about it almost a year later, they were told the patient is no longer welcome there. They asked whether anyone else is seeing this. **If accurate, this affects every practice prescribing stimulants, and it is worth verifying independently rather than taking at face value from one account.**
- **Treatment-resistant severe presentations.** A psychiatrist 25 years post-residency, splitting private practice with per diem psych ER, C/L, and inpatient work, polled the group on what people use when first and second line options have failed across more than two years, two medication classes, and two different therapy providers.
- **Telehealth and suicidality.** A question about managing escalating suicidal ideation in a high-functioning telehealth patient, specifically around patient addresses and what obligations follow. Practical and unresolved.

- **Gender-affirming care conflict.** A case involving a trans teen self-injecting testosterone obtained online, with strongly unsupportive parents and a prior suicide attempt tied to gender dysphoria. Expert input requested.
- **Interventional psychiatry.** Someone asked how to get started with esketamine or IV ketamine, which is a recurring "how do I add this service line" question.
- **Free CME-adjacent resource:** the ISRP monthly Reproductive Psychiatry Peer Consultation Group, free and open to physicians of all specialties, for psychotropic management in pregnancy and postpartum.
- **Scope of practice is politically live.** Two posts on the Lindsey Clancy case, one explicitly pointing out that her outpatient treaters were NPs with full prescribing authority in Massachusetts. Expect this argument to keep surfacing.

Signals worth tracking

Signal	Why it matters	Confidence
Appealing downcoded 99214s reverses them and improves future first-pass payment	Direct revenue, repeatable, one detailed first-hand account with a specific method	Single source, high detail
CVS nationwide UDS requirement before stimulant refills, since Oct 2025	Would affect every stimulant prescription workflow	Single source, unverified, no policy citation
Charm EHR e-claims and PDMP unreliable	Vendor risk if ever evaluated	Single detailed account
Downcoding sentiment escalating toward collective legal action	Suggests this is systemic, not one practice's billing error	Multiple posts, same week

Caveats

- **Comments are not collected yet.** Every post this week shows zero comments because comment scraping was scoped but not implemented. The answers to most of these questions live in the comments, so this digest reports what is being *asked* far better than what is being *concluded*. That is the single biggest upgrade available to this product.
- Everything here is one practitioner's unverified account. Nothing in this digest is a source of truth for billing, compliance, or clinical decisions.
- Referral requests, job postings, and practice advertisements are excluded by design.